NYS Office of Mental Health

ACT ADULT AND YOUNG ADULT STANDARDS OF CARE SURVEY TOOL MAY 2026

Standard of Care Focus	EXEMPLARY (In addition to Core)	CORE (Absence of a preponderance of core indicators may require a performance improvement plan)
		Intake
ACT SOC 1.11 Intake	1. ACT team members engage with a person prior to admission into ACT and well in advance of hospital discharge or jail release assisting with coordination of discharge planning.	1. Following referral, the ACT Team locates individuals and engages them in services. 2. The team makes an admission decision no later than seven (7) business days of initial contact with the individual. ACT Team clearly documents their admission decision and notifies SPOA of their decision. Delays in admission are clearly documented in the individual's record.
		Assessment and Evaluation
ACT SOC 2.11 Initial Assessment	1. The entire team meets to review clinical assessments of new admissions to formulate multidisciplinary team review and to inform planning recommendations. 2. Each individual is screened using the Columbia-Suicide Severity Rating Scale (C-SSRS).	1. An immediate needs assessment and documentation of a plan to address these immediate needs is completed within 7 business days of admission. 2. An Initial Assessment is completed within 60 calendar days of admission to inform the initial Service Plan: a. Staff with appropriate expertise complete the assessments. b. The team reviews and develops clinical formulations under the guidance of the MD, Team Leader or designated clinical supervisor. c. Person-specific risk factors and patterns are identified, as well as effective and ineffective coping strategies, level of independence in critical areas, supports and interventions. d. There is evidence that exploration of personalized recovery goal(s) occurred during the assessment. 3. Standardized screening and assessment instruments are used for: a. Risk b. Substance Use c. Health (reviewed by MD, NP or RN)
ACT SOC 2.12 Ongoing Assessment and CAIRS	1. The team's CAIRS BASF and FUAF data is entered within 2 weeks of the due date. 2. The team uses PSYCKES to identify and monitor individuals who could benefit from clinical review, and inform treatment planning. 3. The team uses RHIOs/QEs to identify ER/hospitalizations.	1. There is evidence that the team enters individual data into CAIRS the Baseline Assessment Form (BASF) is completed within 30 days of admission. 2. 2The CAIRS Follow-Up Assessment Form (FUAF) is updated at least every six months. 3. The FUAF and comprehensive assessment is completed every six months and informs the service plan review. 4. Specific comprehensive assessments are completed as needed, specifically in areas of Risk, SUD, Medical, and Psychiatric.

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Service Plans		
ACT SOC 3.11 Service Plan	1. Individuals develop WRAP plans. 2. The team uses systematic tools to determine readiness for change. 3. The team uses tools that reflect a rehabilitation process: e.g.: identifying a personal goal, assessment of strengths, barriers and strategies, and timelines to achieve the goal. 4. Recipients receiving rehabilitation services actively practice and utilize social and communication skills. 5. Treatment planning and review occurs with family/supports present if the person consents. 6. A copy of the plan is shared with family with consent and medical/other providers where appropriate.	1. Every individual has a person-centered service plan developed in partnership, within 60 days of admission, with documented involvement of the individual, physician and the team leader or designated clinical supervisor involved in the treatment. 2. The plan is signed by the staff person completing the plan and an LPHA on the ACT Team. If the plan is not signed by the individual, the team documents in progress notes that the plan was reviewed and agreed upon by the individual. 3. The service plan reflects: a. A recovery orientation, including life goals, such as educational, vocational, residential, social, or recreational pursuits. b. Evidence of shared decision making with individual, team, family support and/or collateral. c. Activities to be carried out from an AOT order, where applicable. d. Future orientation and documentation of specific and individualized discharge criteria. 4. The plan is shared with individual, and family, medical and other providers, with consent. 5. Clinical interventions are not solely focused on engagement and treatment adherence, but include: a. Building and testing skills, and fostering independence b. Strengthening and building community links and supports 6. There is evidence that the team and individual have identified barriers associated with reaching goals and strategies to assist in overcoming these barriers. 7. At least one of the documented services is a Community Integration and Re-integration service: a. Care coordination;

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		b. Health promotion; c. Comprehensive transitional care from inpatient and ACT to other settings, including appropriate follow-up; d. Individual and family support, which includes authorized representatives; e. Referral to community and social support services, if relevant; f. The use of health information technology to link services, as feasible and appropriate
ACT SOC 4.11 Progress Notes & Service Plan Reviews	1. Contacts are documented in progress notes and signed within 72 hours of the contact taking place.	1. New information, changes in individual's functioning, and difficulty with engagement are clearly identified and immediately communicated to team members via progress notes, logs, verbal reports etc. 2. The service plan is reviewed and revised to include new strategies, services etc. in response to changes in individual's circumstances and functioning. 3. All plans are reviewed at least every 6 months from the time of admission.
Ongoing Care		
ACT SOC 4.12 Ongoing Crisis and Risk Management	1. The team has a QI initiative focused on reduction of harmful behaviors.	1. Each individual has a current crisis/relapse prevention plan, developed with the individual, which is reviewed and/or updated every six months, or more frequently as clinically indicated. 2. Crises are rapidly addressed with input from multiple team members, and strategies for addressing individual difficulties integrated across the disciplines and treatment approaches. 3. The team operates a continuous and direct after-hours on-call system with experienced ACT staff skilled in crisis intervention procedures. 4. The team directly provides in-person after hours services where appropriate. 5. The team maintains a current crisis/on-call log to track after-hours calls, and documents calls and interventions in individual progress notes. 6. When the team identifies individuals who will not come to meet the psychiatrist/NPP, there is evidence that they provide services to that individual in the community in addition to the required community visits as stated in program guidelines. 6. The team utilizes a tracking system to identify and monitor at-risk individuals and documents their interventions in individual progress

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		notes. 7. The team proactively addresses any issues that potentially place the individual at risk, and documents attempts to mitigate.
ACT SOC 4.13 Attention to Co-Occurring Medical Issues	1. There is evidence that the team is helping individuals manage their diabetes, high cholesterol, hypertension, and/or obesity. 2. There is evidence of collaboration and relationship building with community medical providers.	1. The psychiatrist and/or the NPP document the individual's response to medication, symptoms, and any side effects. 2. The record includes any recent hospitalizations and or medical treatments and team's active response. 3. The team documents efforts to coordinate services between health and behavioral health services for individuals.
ACT SOC 4.14 Attention to Co-Occurring Disorders	1. The team is fully based in Co-occurring Disorders treatment principles, with person-centered harm reduction approaches utilized by team staff. 2. Cross-discipline co-occurring treatment is provided by the team.	1. The record includes any recent substance use and the team's active response, including planned interventions. 2. Evidence-based substance use treatment is offered and provided when agreed upon by the individual; individuals with substance use disorders have at least one session weekly in such treatment. 3. The record identifies efforts taken to provide integrated substance use treatment for individuals with co-occurring disorders and the service plan reflects active IDDT treatment. 4. The team facilitates access to self-help services to support the individual outside of the ACT program.
ACT SOC 4.15 Communication with Families/Collaterals	1. The team documents outreach efforts to family/collaterals, including Health Homes, MCOs, and housing programs. 2. The team has one or more contacts per month per individual with their natural support system in the community, including but not limited to family, family of choice, etc.	1. The team identifies the individuals' family and support network in the service plan, with permission from the individual. 2. The team assists to develop natural supports and works with individual's natural support networks and service providers to further integrate into the community. 3. The team demonstrates use of all sources (family, friends, etc.) to contact a missing individual.

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ACT SOC 4.16 Transition Planning	1. There is evidence of a system in place for identifying and practicing recovery skills needed for all ACT enrollees. 2. The team engages individuals in the WSM for Transition curriculum. Each individual leaves ACT with a WSM/or WRAP plan. 3. The team uses a formalized transition model, including specific stages, groups, and strategies.	1. There is evidence that a transition tool is used and individuals who indicate readiness for transition are actively engaged in the transition process. 2. The team establishes a positive expectation for transition for individuals via practice of: a. Introducing the idea of transition at admission. b. Engaging long-stay ACT individuals in transition discussions (progress and goals for the future). 3. Specific activities associated with positive discharges are evident in the record. 4. There is evidence that a system is in place that assures early, and regular post-d/c follow up with new providers and natural supports to ensure that discharge plan is being followed.
Staffing		
ACT SOC 5.11 Staffing	1. Team has a Peer Specialist.	1. ACT team staffing ratios and composition conform to the model, 10:1 (Including Team Leader as 1FTE). 2. Specialists on the team provide targeted interventions and collaborate with other team members. 3. The team has all required roles, as outlined in the NYS ACT Program Guidelines, as part of their ACT Team staff.

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ACT SOC 5.12 Team Protocols/ Functioning	1. The team utilizes TMACT or another tool to assess fidelity to the model.	1. The team conducts face-to-face contacts based on the individuals need, with the minimum number of 2 (two) to 6 (six) face-to-face contacts per month. Justification for fewer than 6 (six) contacts per month is documented. 2. Individuals are seen by at least 3 (three) different ACT Staff each month, as outlined in the NYS ACT Program Guidelines. 3. The team conducts a minimum of 80% of contacts in the community. 4. The team is involved in at least 80% of hospital admissions, discharges, and ER visits. 5. The Physician and NPP conduct visits in community as per the guidelines. 6. Daily team meetings occur at least 4 (four) days a week and the team reviews the status of each individual and plans for response to potentially elevated risk status are formulated. 7. Communication boards, logs, and other devices are maintained as per the guidelines.
ACT SOC 5.13 Training	1. The agency has a system in place for assessing staff competencies. 2. The team has a system in place for reviewing the team profile, including FIT data, and identifies individual staff training needs. 3. The team maintains a fully trained staff as evidenced by the team Certificate for Training Excellence issued by the ACT Institute twice per year.	1. The team provides or arranges for training in the ACT model and relevant Evidence-Based Practices for each specialist. 2. Required core training for all new hires is completed according to the training requirements (or as indicated on the ACT Institute website). 3. The Team Leader has a system in place for tracking the training status of all staff members. 4. Supervision is routinely provided to all staff.

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Organizational Practices and the Team Operations		
ACT SOC 6.11 Governing Body and Quality Assurance	1. The agency Governing Body (GB) has a subcommittee responsible for overseeing quality improvement initiatives or attending to serious issues raised by external review agencies, individuals, or families. The subcommittee meets at least quarterly or more often as needed 2. There is evidence that the GB's quality assurance activity has reviewed data from the team Profile (comparison data across the state, region, county, etc.) and provided assistance, as needed, to the ACT team to improve performance	The agency has a systemic approach for self-monitoring and ensuring ongoing quality improvement (QI) for the ACT team: 1. The agency's QI plan involves reviewing UR findings and recommendations. Information is used to measure goal achievement, length of stay, barriers to treatment, etc. and incorporated into the team's overall QI plan 2. An agency-wide process for reviewing complex, high risk, high need cases is evident. Individuals with clinical expertise who can provide recommendations on treatment strategies are available to the ACT team. 3. The agency addresses vacancies through an effective coverage plan and active recruitment.
ACT SOC 6.12 Incident Management and Reporting	1. The team uses NIMRS data in conjunction with its incident trending data as it develops and implements preventive measures. 2. All staff receive training on the definition of incidents and reporting procedures, are informed about the Incident Review Committee (IRC) process and the importance of risk management in maintaining safety and improving services at the time of hire and on at least an annual basis.	1. All staff receive training on the definition of incidents and reporting procedures, are informed about the Incident Review Committee (IRC) process and the importance of risk management in maintaining safety and improving services at the time of hire and on at least an annual basis. 2. The agency has an Incident Management policy consistent with NYCRR Part 524 and the Justice Center and conforms to the reporting and follow up requirements of each. 3. The IRC reviews incidents, makes recommendations, and ensures implementation of action plans with the team's administrator. 4. The IRC membership composition is appropriate; members meet the qualifications and are properly trained. 5. The committee compiles and analyzes incident data for the purpose of identifying and addressing possible patterns and trends.
ACT SOC 6.13 Cultural Competence	1. The team utilizes a Diversity and Inclusion standardized self- assessment and uses the results in developing policies and trainings. 2. The team is trained in the use of the Cultural Formulation interview (DSM 5). 3. The team includes bilingual staff representative of the individuals enrolled.	1. Diversity, inclusion, equity, or cultural/linguistic competence plan that includes: a. Workforce diversity (data informed recruitment to hire, train and retain staff and supervisors who are from the most prevalent cultural groups of its ACT individuals served); b. Workforce inclusion; c. Reducing disparities in access, quality, and treatment outcomes; 2. Solicit input from diverse community stakeholders/organizations. The plan ensures that language assistance services are available.